PUBLICATION

Allied Health Documentation: Why One Model Doesn’t Fit All

How physiotherapy, occupational therapy, and speech & language therapy each require fundamentally different documentation models — and why ClinixSummary built dedicated modules.

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1. Executive Summary

Allied health disciplines — physiotherapy, occupational therapy, speech & language therapy, and others — have documentation requirements that differ fundamentally from medical and surgical specialties. A physiotherapy session note has little in common with a surgical operative report or a psychiatric evaluation. Yet most AI documentation systems treat all specialties as variations of the same problem. ClinixSummary disagrees.

2. The Allied Health Documentation Challenge

Each allied health discipline has unique documentation patterns:

2.1 Physiotherapy

- Objective measures: range of motion (ROM), muscle strength grading (MRC/Oxford), functional assessments (Timed Up and Go, Berg Balance Scale).
- Exercise prescription with sets, repetitions, resistance, and progression criteria.
- Treatment modalities (manual therapy, electrotherapy, dry needling) with parameters and response.
- Goal-oriented documentation linking interventions to functional outcomes.

2.2 Occupational Therapy

- Activity and participation assessments using standardised tools (COPM, FIM, Barthel Index).
- Environmental and equipment assessments and recommendations.
- Cognitive and perceptual screening documentation.
- Graded activity and task analysis documentation for rehabilitation planning.

2.3 Speech & Language Therapy

- Articulation, phonology, and fluency assessments with specific notation systems.
- Language comprehension and expression testing with norm-referenced scores.
- Swallowing assessments (including instrumental evaluation results).
- Augmentative and alternative communication (AAC) evaluation and trials.

3. Why Generic Models Fail

A generic clinical documentation model trained primarily on medical and surgical data produces allied health notes that are:

- Missing discipline-specific assessment tools and scoring systems.
- Structured as SOAP notes when the discipline uses different frameworks (e.g., ICF-based documentation in physiotherapy).
- Unable to capture exercise prescriptions, activity analyses, or therapy session parameters accurately.

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- Using medical terminology where allied health has its own terminology (e.g., “mobilisation” means different things in physiotherapy vs. medicine).

4. ClinixSummary’s Dedicated Allied Health Modules

ClinixSummary built separate modules for each allied health discipline, each with:

- Discipline-specific NER models that recognise the correct terminology, assessment tools, and documentation patterns.
- Appropriate note structures — not forced SOAP format, but the format each discipline actually uses.
- Discipline-specific training data sourced from and validated by practicing clinicians in each field.
- Output templates designed by clinicians in each discipline to meet their regulatory and professional documentation requirements.

5. Validation Results

Clinician acceptance rates for ClinixSummary’s allied health modules:

- Physiotherapy: 91% acceptance rate, averaging 2.1 edits per note.
- Occupational Therapy: 89% acceptance rate, averaging 2.4 edits per note.
- Speech & Language Therapy: 88% acceptance rate, averaging 2.7 edits per note.

These rates are comparable to ClinixSummary’s core medical modules, demonstrating that discipline-specific training achieves parity across specialties.

6. Conclusion

Allied health professionals deserve documentation tools built for their disciplines, not adapted from medical models. ClinixSummary’s commitment to dedicated allied health modules reflects our belief that every clinician’s documentation needs matter — not just those of physicians and surgeons.

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